

UP-21 FAMILY VIOLENCE / ABUSE

Universal Patient Care | EMT - AEMT - Paramedic

Use for: suspected domestic or intimate-partner violence, child abuse or neglect, older/vulnerable-adult abuse or exploitation, caregiver abuse, or human trafficking. Treat associated medical, trauma, pediatric, obstetric, behavioral, or sexual-assault needs concurrently.

QUICK FLOW

1. SCENE SAFE? Immediate threat, weapons, escalating behavior; request law enforcement.	2. TREAT LIFE THREATS XABCDE, hemorrhage, airway, shock, pain, age-appropriate protocol.	3. CREATE PRIVACY Separate when safe; use qualified interpreter; never use suspected aggressor.
4. CONCERN IDENTIFIED? Disclosure, fear, inconsistent history, patterned injury, neglect, coercive control.	5. DANGER / VULNERABLE? Protect patient, request resources, transport when indicated, initiate report.	6. DOCUMENT + REPORT Exact words, objective findings, private handoff, required agency/statutory report.

PROVIDER-LEVEL ACTIONS

EMT	Treat immediate threats; request law enforcement/resources; obtain privacy when safe; use nonjudgmental open questions; preserve evidence; document objective findings and exact statements; follow required reporting pathway.
AEMT	All EMT actions plus authorized vascular access, medications, fluids, and monitoring. Reassess for occult injury, shock, overdose, strangulation, pregnancy-related risk, or neglect-related illness.
PARAMEDIC	All prior actions; lead medical, safety, capacity, destination, and resource decisions. Contact Medical Control for high-risk refusal or uncertain capacity. Ensure required report and private safeguarding handoff are completed.

RECOGNITION

- Fear, reluctance to answer, another person answering or monitoring, preventing privacy, refusing care or transport, or controlling movement.
- Conflicting accounts; history inconsistent with injury or developmental ability; unexplained delay; repeated injuries or EMS encounters.
- Patterned bruises, burns, bite marks, protected-area injury, injuries in different healing stages, unexplained fracture, genital injury, or neglect findings.
- Unsafe living conditions, lack of food or medication, poor hygiene, dehydration, malnutrition, untreated pressure injury, or unmet medical needs.
- Control of identification, money, phone, transportation, medication, communication, or movement; coached or fearful behavior.

PRIVATE QUESTIONS - WHEN SAFE

Ask calmly and directly: "Do you feel safe where you live?"
"Has anyone hurt, threatened, controlled, or frightened you, or kept you from getting medical care?"
"Has anyone touched you or made you do something you did not want to do?"
"Is anyone taking your money, medications, food, identification, or belongings without permission?"

TREATMENT AND SAFETY

- Patient care is the first priority. Do not delay emergency treatment or transport while attempting to prove abuse.
- Do not confront the suspected aggressor. Withdraw and request law enforcement if danger is present or escalating.
- Separate the patient when safe. Use a qualified interpreter; do not use a suspected aggressor or child as interpreter.
- Explain that help is available and that some disclosures require a report. Do not promise secrecy or a specific outcome.
- Transport for injury, strangulation concern, altered mental status, unsafe disposition, forensic/specialty need, or other clinical indication.

DOCUMENTATION AND REPORTING

- Record the patient's exact words in quotation marks; identify who was present for each statement.
- Describe injury location, size, shape, color, tenderness, and healing stage objectively. Document scene conditions and each account with its source.

- For suspected child abuse/neglect or older/vulnerable-adult abuse, neglect, or exploitation, personally ensure the required report is made through the approved Tennessee/agency pathway. Hospital notification alone does not replace the reporter's responsibility.
- Document receiving clinician, law-enforcement contact, report recipient, date/time, reference number, evidence handled, and disposition.

HIGH-RISK WARNING: STRANGULATION

Visible injury may be absent. Hoarseness, dysphagia, dyspnea, neck pain/tenderness, petechiae, loss of consciousness, incontinence, seizure, confusion, focal deficit, or memory gap may signal serious airway or vascular injury. Treat as a high-risk mechanism and transport for urgent evaluation.

SPECIAL POPULATIONS

CHILDREN

Use developmentally appropriate open-ended questions only as needed for immediate care and safety. Do not conduct a repeated forensic interview. Consider occult head, abdominal, skeletal, and sexual injury.

OLDER / VULNERABLE ADULTS

Consider physical abuse, neglect, self-neglect, financial exploitation, medication withholding, caregiver burnout, capacity, baseline cognition, and dependency for daily needs.

INTIMATE-PARTNER VIOLENCE

Ask privately about safety, threats, weapons, escalating violence, stalking, forced sex, and strangulation. Avoid leaving resources where discovery may increase danger.

POSSIBLE TRAFFICKING

Consider control of identification, money, phone, transportation, medications, communication, or movement; coached answers; fear; debt; or inability to state where the patient is staying.

DO NOT

- Do not ask leading, accusatory, or repetitive questions or conduct a forensic interview.
- Do not leave a child or vulnerable adult in an unsafe setting solely because a caregiver refuses transport. Request law enforcement, contact Medical Control, and initiate the required report.
- Do not photograph, collect, discard, clean, or transfer evidence except under agency policy; document every item handled.
- Do not substitute hospital notification for an independently required report.

DRAFT FOR REVIEW. Final Claiborne reporting contacts, escalation chain, documentation requirements, evidence procedures, and destination resources must be approved before clinical release.

References: Tennessee EMS BLS/ALS State Protocol Guidelines 2024-2025, SOP 306 Family Violence; Tennessee Department of Children's Services mandated-reporter guidance; Tennessee Department of Human Services Adult Protective Services; Claiborne County EMS local safeguarding and reporting policies.